

WEBSITE LAUNCH PROGRAM | REVISION 3.0

Facts and Approvals Still Required

What the website still needs from AirEvac leadership, aviation operations, clinical leadership, and legal. The build work is complete. Everything below is information or a sign-off that only AirEvac can give, and the site deliberately withholds the related content until it arrives. Prepared 30 July 2026.

Read this first

AirEvac approved "all changes needed to make the site as AI searchable and recommendable as possible", and those changes are built and live on the development site. That approval covered **decisions**.

It does not, and cannot, supply **facts**. The website is built so that no claim about an accreditation, an aircraft, a clinical capability, or an operating authority can appear unless the supporting document is on file, with an owner, an approval date, and an expiry date. That rule is enforced by the software itself, not by anyone remembering to check. It is the reason the site can be trusted, and it is why a general approval does not turn into published claims.

So the items below are not requests for permission. They are requests for a document, a name, or a list.

1. The single most valuable item on this page

EURAMI accreditation certificate

The website currently publishes no accreditation at all. Not on the credentials page, not in the data search engines read, not in the file that AI systems read.

This is not because anything is wrong. The EURAMI record is already in the system with the exact accredited scope, the expiry date of 25 August 2027, and a public link anyone can verify it against. One field is empty: the date a named AirEvac approver confirmed the certificate itself is on file. The software will not publish an accreditation on the strength of a directory entry alone.

WHAT THIS COSTS RIGHT NOW: an accreditation from a named body, with a verifiable third-party link, is the strongest credibility signal this website could carry. It is exactly what an AI system weighs when deciding whether to recommend a medical transport provider, and exactly what a hospital case manager looks for when choosing between providers. It is currently invisible.

What we need: the EURAMI certificate document, and confirmation of who at AirEvac approves its publication. Publication follows the same day.

2. Facts that unlock withheld content

#	What we need	What is happening without it
F1	Clinical reviewers. The names, roles, and credentials of the medical staff who will be publicly named as having reviewed the medical pages, and their agreement to be named.	Every medical page displays a visible "Under review, not yet signed off by a qualified reviewer" banner. Search engines and AI systems see no named medical authority behind the clinical content, which is a significant credibility gap in this industry.

#	What we need	What is happening without it
F2	Online profiles. The exact current web addresses of AirEvac own profiles and directory listings: social accounts, industry directories, airport tenant listing.	Search engines cannot confirm that the website, the business profile, and the directory entries are the same organization. This is one of the strongest signals available and it is currently empty. We did not guess from the old website, because pointing at a defunct account is worse than pointing at nothing.
F3	Route detail confirmation. An operations read-through of the twelve route pages: Cancun, Cozumel, Los Cabos, Puerto Vallarta, Bahamas, Dominican Republic, Jamaica, Turks and Caicos, Cayman, Belize, Costa Rica, Honduras.	Each page describes ground transfer times, local hospital and discharge realities, and border paperwork. It was drafted from general regional knowledge, not from AirEvac case files. It is live and hedged throughout, but an operations lead should confirm it. Corrections take effect the same day.
F4	Aviation facts. Certificate-holder wording, whether the certificate number may be published, operating bases, and aircraft range wording.	The site describes AirEvac as coordinating transport directly with no broker in between, and says nothing about operating authority. That is accurate and deliberately limited.
F5	Photography. Aircraft, crew, and facility images AirEvac has permission to publish, with anyone identifiable in them cleared.	Current images came from the existing website. Better photography improves every page it appears on, and it is one of the few remaining things that would visibly lift the site.
F6	Service area boundaries. Confirmation of which countries and regions are routine coverage versus case-by-case.	The site publishes four regions and states that other destinations are reviewed case by case.

3. Approvals tied to launch

These do not block the development site. They block going public.

- **Legal sign-off on the privacy notice, the Notice of Privacy Practices, and the terms of use.** All three are written and all three currently display an "under review" banner. That banner is correct and should not be removed to make the site look finished.
- **A billing and legal decision on patient cost notices.** The previous Patient Rights and Cost Information page was removed by the July change handoff. Its content is archived and can be restored. Someone should confirm no notice is legally required before launch.
- **Operations approval of the 90-minute response wording** in the service questions, and of the **Price Lock Guarantee** wording on the private pay page. Both are published as written in the July handoff.
- **Privacy approval of the email and fax route for clinical documents.** The site instructs people to email records to ops@aeiamerica.com or fax them. Someone should confirm how those are handled and retained once received.
- **Operations approval of the public fleet statement**, "two Learjet 31A aircraft". No registration numbers appear anywhere on the site.

4. Live chat: decisions only AirEvac can make (new in revision 3.0)

The chat is built and has been used end to end on the preview. What is below is not development work; it is four decisions and one rewrite that the build team cannot make on AirEvac's behalf.

- **The privacy notice has to be rewritten to describe live chat.** The current notice tells visitors the website receives no clinical information. A chat makes that untrue within the first minute of the first real conversation. **This is the hard gate: chat cannot face the public until the notice matches what the site actually does.**
- **How long transcripts are kept.** The build uses 30 days as a placeholder. The privacy officer should set the real number, because the privacy notice cannot state a period AirEvac has not approved.
- **Who staffs it, and when.** Chat is offered only while a coordinator is signed into the console with availability switched on. When nobody is, the widget says so and shows the phone number rather than leaving somebody waiting. That is honest, and it also means an unstaffed chat is worth very little. The blueprint asked for a 30-day staffing trial before chat became customer-facing.
- **Whether coordinators may discuss clinical detail in chat.** The site currently tells visitors not to send records through it and directs them to email or fax. Clinical leadership should confirm that is the intended boundary.
- **Which operations address receives the transcript notice.** Every conversation ends with a notification for the record. It carries a reference number and no conversation text. It is pointed at a testing address today and must move to a real operations mailbox before launch.

5. Two things the website itself still needs before launch

Listed here because they are business decisions, not build work:

- **The transport request form does not deliver anywhere yet.** It issues a reference number and records that a request arrived, but it does not send the request to anyone, because no approved system has been chosen to receive it. Today the phone line is the real channel. Before launch, either choose that system or remove the form and lead with phone and email. A form that silently reaches nobody is worse than no form.
- **Domestic United States service page.** The site lists the United States as a service area but has no page describing domestic-only transport, because we do not know whether AirEvac accepts those cases. If you do, tell us the typical case, what is coordinated, and any limits, and we will build the page. If not, we leave it out. It is meaningful search demand either way.

6. What no website can do

The AI searchability specification is honest about this and so are we. The website can be found, understood, and cited. It cannot manufacture the reasons an AI system or a hospital chooses to recommend AirEvac. Those come from outside the website:

- **Independent listings** that confirm AirEvac exists where it says it does: the airport tenant directory, industry directories, the accrediting body provider listing.
- **References.** Hospitals, case managers, cruise lines, and assistance companies willing to be named in writing as referring partners.
- **Press.** Local business coverage and medical transport trade press, tied to real events.
- **Reviews.** A policy for inviting genuine client reviews on Google. Never filter them, never offer anything in exchange: both violate platform rules and federal advertising guidance, and both are detectable.
- **Answering the phone well.** A fast, clear 24/7 response remains part of whether an enquiry becomes a transport, and no amount of website work substitutes for it.

7. How to respond

Reply by email with the item number and your answer, document, or list. Items can come back one at a time; nothing waits on the others. The EURAMI certificate in section 1 has more effect than everything else on this page combined, so if only one thing gets done, make it that one.